Utah Department of Commerce
Office of Professional Licensure Review

2025 Periodic Review

Respiratory Care

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Executive Summary

Background

Respiratory care practitioners (RCPs) are healthcare professionals educated and trained to test and evaluate, assist in the diagnosis of, and treat conditions related to the cardiopulmonary system. They work with critical care patients and individuals with chronic conditions to assist with their breathing.

Utah requires RCPs to be licensed. Licensure requirements, including graduation from an accredited respiratory care program and passing an examination, are relatively standard throughout the U.S. Most states (40), including Utah, allow RCP applicants to qualify by obtaining the Certified Respiratory Therapist credential (CRT), while the other 9 states that license RCPs require the more advanced Registered Respiratory Therapist credential (RRT). During the Covid-19 pandemic, Utah temporarily issued RCP apprentice licenses, allowing students near the end of their program to work in a clinical setting under indirect supervision.

Regulatory Model Recommendation: *Continue to license RCPs*

- RCPs provide advanced care that has the potential to cause serious and permanent harm, including infection, lung damage, and death. They almost always work with vulnerable patients who require assistance in order to breathe.

Recommended Regulatory Adjustments: *None*

- Given indications of relatively safe practice, OPLR does not recommend increasing entry requirements for the RCP profession.
- While RCP access may be constrained in certain parts of the state, particularly rural areas, Utah's entry requirements are in line with other states. Due to the potential for harm, lowering entry requirements is not advisable.

Additional Recommendation: *Reinstate the respiratory care apprentice license*

- DOPL did not receive any complaints against RCP apprentices during the time the license was being issued.
- At least 17 other states have some kind of license or permit that allows RCP students to work while still in school.
- The apprenticeship may increase access by allowing students to begin offering services, consistent with their training, before licensure and may improve safety by increasing the level of experience that students graduate with.
- OPLR recommends strict safety guardrails, including: limiting apprentices' scope of practice to patients in non-critical conditions and prohibiting the performance of mechanical ventilation.

Context

Consistent with its legislative mandate,[1] the Office of Professional Licensure Review (OPLR) reviewed Utah's licensing laws for respiratory care practitioners. The review evaluated how well current regulations:

1. Protect the public from present and consequential physical and financial harm
2. Balance public and practitioner access to the occupation
3. Limit the economic impact of regulation on consumers, practitioners and the state[2]

OPLR's research for this review included analysis of Utah's current laws and rules, licensing and complaint data from the Division of Professional Licensing (DOPL), licensee survey results, academic literature, health workforce data, as well as laws and policies in other states. OPLR also conducted interviews and focus groups with employers, educators, and DOPL employees. See Appendix 1 for more information.

Background

Profession Overview

A respiratory care practitioner (RCP), commonly referred to as a respiratory therapist, is a healthcare professional educated and trained to test and evaluate, assist in the diagnosis of, and treat conditions related to the cardiopulmonary system. RCPs treat both patients who are in critical condition, such as those on ventilators, and those with chronic cardiopulmonary disorders, such as asthma and chronic obstructive pulmonary disease (COPD). Their work includes direct physical contact with patients and the management of machines to assist with breathing.

Profession in Utah

Respiratory care is a licensed profession in Utah. At the time of OPLR's review there were 1,981 RCPs in the state.[3] To become licensed as an RCP in Utah, an individual must have a high school diploma or equivalent, graduate from an accredited respiratory care program,[4] and pass either the exam to qualify as Certified Respiratory Therapist (CRT) or the additional exam to qualify as a Registered Respiratory Therapist (RRT) (see Appendix 2.1).[5] More than half of RCPs licensed in the state qualified with an associate degree, while just over a third qualified

[1] UCA 13-1b-203(2)
[2] UCA 13-1b-302
[3] DOPL Licensee Data, accessed January 2025
[4] The Committee on Accreditation for Respiratory Care (CoARC) is the accrediting body and currently accredits both associate and bachelor's level programs; CoARC Accreditation Policies and Procedures Manual (2025).
[5] UCA 58-57-4; R156-57-302a; R156-57-302b; The exams to qualify for the CRT and RRT credentials are administered by the National Board for Respiratory Care.

with a bachelor's. Most have attained the RRT credential.[6]

RCPs in Utah must renew their license every two years.[7] However, they are not required to complete continuing education (CE) credits for renewal, and there are no provisions in statute or rule that make renewal contingent on maintaining the CRT or RRT credential associated with the approved entry examinations.[8]

The scope of practice for RCPs in Utah includes "treatment, operation of equipment, management, diagnostic testing, and care" related to the cardiopulmonary system under the supervision of a prescribing practitioner.[9]

During the Covid-19 pandemic, DOPL used emergency powers given to it in law to create and issue respiratory care apprentice licenses.[10] This allowed respiratory care students to work under indirect supervision during the final year of their program.[11] The apprentice license was temporary and was discontinued in 2023. Between December 2020 and May 2023, DOPL issued apprentice licenses to 210 individuals, 184 of whom went on to gain full RCP licensure. Of RCPs licensed after 2021, about 15% were apprentices first.[12]

Approaches in Other Jurisdictions

Forty-nine states license RCPs, Alaska being the exception. Entry requirements appear relatively standard between states.The main distinction is whether states allow RCPs with only the CRT credential to become licensed. Most states (40, including Utah) allow licensure with the CRT credential, eight require the RRT credential, and one has separate licenses for the two credentials.[13] Utah is one of only six states that do not require licensees to complete CE credits for renewal. The states that do require CE credits mandate between six and fifteen hours on an average annual basis.[14]

Regulatory Model Assessment & Recommendation

Framework

In an effort to standardize how appropriate regulatory models are determined for each profession (e.g. license, registry, no regulation, etc.), OPLR developed a framework which

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[6] DOPL Respiratory Care Practitioner Licensee Renewal Survey, July 2024; 56% of respondents qualified for licensure with an associate degree and 38% qualified with a bachelor's degree. Eighty-eight percent (88%) have the RRT credential.
[7] UCA 58-57-6
[8] UCA 58-57-6; R156-57-303
[9] UCA 58-57-2(3)
[10] UCA 58-1-307(4)
[11] OPLR obtained a copy of the temporary respiratory care apprenticeship license application form used during the Covid-19 pandemic.
[12] DOPL Licensee Data
[13] OPLR Policy Scan
[14] OPLR Policy Scan

incorporates its statutory review criteria.[15] Appropriate models are determined principally by an evaluation of the potential for harm and related factors that may aggravate or mitigate the potential for harm. These factors include the availability of consumer choice, vulnerability of patients, and independence of practice. See Appendix 3.1 for potential regulatory models and the factors in OPLR's framework.

Potential for Harm

The work that RCPs perform has the potential to cause severe harm. Many of the services RCPs provide, such as intubation, are physically invasive and can cause harm if performed by someone who is not trained. Additionally, improper management of a ventilator, which is likely without proper training, could result in damage to the lungs, infection, or even the death of a patient.[16] The U.S. Department of Labor's O*Net Consequence of Error Ranking, which estimates the severity of harm that could arise from different professions, places an RCP's consequence of error as an 88 out of 100, even higher than registered nurses.[17] When a profession has a high likelihood of harm, including severe and permanent harm, when performed by an untrained individual, the state should regulate entry and practice requirements to protect the public.

The potential for harm associated with respiratory care is also related to the vulnerability of the patients RCPs tend to work with. RCPs overwhelmingly work in hospitals, where patients are likely sick or injured and vulnerable to further harm.[18] Critical care patients often cannot breathe on their own, and proper respiratory care is vital for survival. Even patients that are not in critical condition, such as those with asthma or COPD, require care to maintain their quality of life and prevent more serious health issues. Additionally, RCPs work with elderly patients in home health and long term care settings.[19]

Related Harm Factors

Factors that exacerbate potential for harm from respiratory care services include a lack of consumer information and choice. Patients receive very little information about their RCP, and do not select their RCP. Patients are assigned an RCP by the healthcare facility. Because regulation through market forces requires those seeking a service to be able to choose their service provider based on adequate information, higher-risk professions, such as respiratory

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[15] Among other criteria, OPLR is required to evaluate "whether the regulation of the occupation is necessary to address a present, recognizable, and significant harm to the health, safety, or financial welfare of the public" and consider "potentially less burdensome alternatives to the… existing regulation; UCA 13-1b-302

[16] Harabhai & Mahboobi (2025)

[17] O*Net Consequence of Error Ranking

[18] DOPL Respiratory Care Practitioner Licensee Renewal Survey, July 2024; 79% of RCPs who report working in Utah do so in a hospital setting (65% in inpatient, 7% in emergency departments, and 6% in outpatient).

[19] DOPL Respiratory Care Practitioner Licensee Renewal Survey, July 2024; 8% of RCPs who report working in Utah do so in a nursing home or other long term care setting and 3% report working in home health.

care, that do not allow for patient choice require state intervention to help protect patient safety.

Potential harm is mitigated, in part, by the fact that RCPs work in care teams and are likely surrounded by providers who can help monitor their care and step in when there are issues. However, RCPs can interact with patients without direct supervision, giving them some autonomy in the moment, and some RCPs work in settings that put them in more independent situations, such as home health and hospice.[20]

For more detail on OPLR's analysis of respiratory care according to the framework, see Appendix 3.2.

Recommendation

After weighing these different factors, OPLR concludes that the profession of respiratory care warrants continued licensure.

Regulatory Model Adjustments & Recommendations

Framework

After determining an appropriate regulatory model, OPLR's framework also evaluates whether adjustments should be made within a recommended model to address any material and existing safety and access issues affecting the Utah public and practitioners. Regulatory model adjustments may include changing entry qualifications, the scope of practice, unprofessional or unlawful conduct, and/or supervision and independence provisions (see Appendix 4.1).

Safety Issues

OPLR found little to no evidence suggesting that RCPs in Utah are practicing in an unsafe manner. Between 2017 and 2022, DOPL received only 14 substantiated complaints against RCPs, for a rate of 0.5 complaints per 100 practitioners. This is the lowest rate of any of the nursing and related professions reviewed by OPLR this year.[21] Additionally, in a survey of nine rural hospital administrators, only two expressed any concern regarding the care offered by RCPs. The feedback was not expressed in a way that indicated serious concern specific to RCPs.[22] This data suggests that Utah's entry requirements are sufficiently rigorous to support competence. See Appendix 4.2 for information on how OPLR uses DOPL complaint data.

[20] DOPL Respiratory Care Practitioner Licensee Renewal Survey, July 2024; only 3% of RCPs working in Utah reported working in home health and hospice.
[21] DOPL Complaint Data; Comparisons across professions are not definitive. Scopes of practice vary across professions and RCPs have a narrower scope than other nursing related professions. Additionally, DOPL complaints only capture a segment of issues that occur in practice.
[22] OPLR Survey of Rural Hospital Administrators, July 2025.

Access Issues

Patient access to RCPs appears to be broadly sufficient. Between 2014 and 2024, the state saw a 5.5% annualized growth rate of RCPs, which outpaced population growth (see Appendix 4.3).[23] Supply and demand modeling from the Federal Health Resources Service Administration (HRSA) predicts that Utah currently has more RCPs than needed, though supply is predicted to drop below demand by 2036.[24] Additionally, Utah's RCP workforce appears to be relatively active in the labor market. The vast majority of licensees are working as RCPs, and of those currently working in Utah, a majority work 33 hours or more a week.[25] Additionally, most plan to continue working their current capacity or to increase work hours, while only a small segment plan to retire in the next two years.[26]

While supply may be sufficient at a statewide level, there is evidence that some areas, particularly rural areas, struggle to fill respiratory care positions. OPLR heard from several rural hospitals that it is hard to compete against urban areas for RCPs.[27] All five of the rural administrators OPLR surveyed that use RCPs said it is 'very difficult' to hire them.[28] Additionally, employment data indicates that rural areas in the state have significantly fewer RCPs per capita than urban areas.[29]

The RCP profession is largely accessible for potential practitioners. There are currently six public RCP programs in Utah (three associate and three bachelor's), and Snow College is setting up a new associate degree program.[30] The average in-state tuition is roughly $13,500 for an associate program and $26,000 for a bachelor's program.[31] A recent licensee survey indicates more than a third of Utah RCPs (38%) graduated with no debt, and almost two-thirds (64%) graduated with $20,000 or less.[32] NBRC exam fees and DOPL licensing fees are modest.[33] Given a median salary of almost $79,000 in Utah, RCPs in the state are likely able to

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[23] DOPL Licensee Data; Utah experienced 1.9% annualized population growth between 2014 and 2024 (Public Health Indicator Based Information System, DHHS).
[24] Health Resources Service Administration Workforce Projections; Utah is one of 22 states/territories with a predicted adequacy rate over 100% in 2025.
[25] DOPL Respiratory Care Practitioner Licensee Renewal Survey, July 2024; Some Utah licensees currently working do not work in Utah (23% work outside Utah, 13% work in an unknown state, based on their primary practice address).
[26] DOPL Respiratory Care Practitioner Licensee Renewal Survey, July 2024; only 2% of respondents expressed plans to retire in the next two years.
[27] OPLR Interview Series and Rural Hospital Focus Group, July 2, 2025.
[28] OPLR Survey of Rural Hospital Administrators, July 2025; No other role was rated 'very difficult' to hire as often as RCPs..
[29] OPLR analysis of data from U.S. Bureau of Labor Statistics & U.S. Census Bureau; Salt Lake has an estimated 42 RCPs per 100,000 people, while Eastern Utah has an estimated 20 per 100,000.
[30] Weber State University, Utah Valley University, and Utah Tech University offer bachelor's degrees, and Weber State University, Utah Valley University, and Salt Lake Community College offer associate programs. Snow College announced the development of an associate program in 2024.
[31] OPLR Analysis of Utah RCP Program Costs
[32] DOPL Respiratory Care Practitioner Licensee Renewal Survey, July 2024
[33] Registered Respiratory Therapist (RRT) | Exam FAQs," NBRC; The NBRC charges $190 for the Therapist Multiple Choice Examination, which can qualify someone as a CRT. Those who want to get an

absorb these costs.[34]

Discussion

A lack of safety concerns with Utah RCPs suggests that Utah's entry requirements are sufficiently rigorous. Limited evidence (i.e., from small studies) suggests that RCPs with a higher level of education, or that hospitals with a higher rate of RRT-credentialed RCPs, may produce better patient outcomes.[35] However, OPLR does not recommend requiring the RRT credential for licensure. Most Utah RCPs choose to get the RRT.[36] Supplementing this workforce with CRT-credentialed RCPs likely helps meet demand without significantly impacting safety. Additionally, hospitals are free to adjust their hiring and workforce practices as they see fit. Hospitals can choose to only hire RRTs, incentivize CRTs to become RRTs, or to adjust the scope or supervision of a CRT-credentialed RCP if they feel it is necessary.

Access issues in rural areas could indicate the need to lower entry requirements. However, Utah already allows those who hold only a CRT credential to become licensed. Many of the states that license RCPs (40) have this same entry requirement, but none require anything less.[37] Additionally, given the critical condition of many patients cared for by RCPs, lowering entry requirements is not advisable. Access to RCP services could be increased by allowing RCP students to perform services they have already been trained on (see 'Additional Recommendations' below). Additionally, rural access to RCPs is being addressed by legislative funding for the new RCP program at Snow College mentioned earlier.

Recommendation

OPLR does not recommend changes within the current licensing model for RCPs.

Additional Recommendations

Reinstate the Respiratory Care Apprentice License

OPLR recommends reinstating the respiratory care apprentice (RCA) license by officially codifying it in law. This license would allow a student enrolled in an accredited respiratory care program to work under the indirect supervision of a licensed RCP. To ensure patient safety: 1) apprentices should only be allowed to treat patients who are not in critical condition, 2) they should not be allowed to perform mechanical ventilation, and 3) they should only be allowed to perform services for which they have received training, as attested by their education program.

RRT must also pass the Clinical Simulation Examination, which costs an additional $200. DOPL Licensing Fee Schedule; DOPL charges $60 for an initial application and $52 for a renewal application.

[34] "Respiratory Therapists," Utah Department of Workforce Services

[35] Kaur et al. (2023); Gresham-Anderson et al. (2021) compared patient outcomes for four hospital groups with high rates of RRT-credentialed RCPs with national averages. One hospital with 99% RRT-credentialed RCPs showed significantly better patient outcomes. The other three did not.

[36] DOPL Respiratory Care Practitioner Licensee Renewal Survey, July 2024

[37] OPLR Policy Scan

As was the case previously, OPLR recommends allowing students to qualify for the apprentice license during the final year of their program. To encourage the transition to full licensure, an apprentice license should expire on the earliest of issuance or denial of a full respiratory care practitioner licensure, loss of student status, 60 days after graduation, or at notice of a failure of the required exam.

OPLR's recommendation is based on multiple factors: the impact an RCA license could have on access to services, demand for RCAs from employers, the safe care provided by RCAs during the COVID-19 pandemic, the impact an apprenticeship can have on an individual's preparedness for practice, and the modest burden it would place on DOPL.

The reintroduction of the RCA license has the potential to increase access to services by allowing individuals to begin offering services earlier in their career, expanding the pool of workers beyond those who are fully licensed. Hospitals can use RCAs for more routine services, freeing up fully-licensed RCPs to focus on more specialized tasks.

There is demand for RCAs from some healthcare providers. In a focus group of rural hospital administrators, several participants expressed strong support for an RCA license. This was based on the difficulty they experience hiring RCPs and their overwhelmingly positive experience working with registered nurse apprentices.[38] Additionally, at least 17 other states offer a license or certification that allows respiratory care students to practice in a limited capacity, indicating some demand for these apprentices in the healthcare industry.[39] A member of the Kentucky Board of Respiratory Care told OPLR that hospital associations are supportive of their state's apprenticeship program and estimated that a majority of students request the apprentice license.[40]

RCAs also appear to deliver safe care. During the time in which RCA licenses were issued, there were no substantiated complaints brought to DOPL against them.[41] Respiratory care leaders from a major employer in Utah did express concern that some hospitals allowed RCAs to perform complex care tasks that they were not prepared for during the pandemic. However, they felt that if the scope of practice for an RCA was clearly defined and did not include high-risk, complex tasks, these issues could be avoided.[42]

Allowing students to practice in a limited capacity before achieving full RCP licensure can help smooth the transition from school to workforce. Apprenticeship experience helps recent graduates become proficient more quickly.[43] Additionally, health care facilities may find it easier to retain an RCP who has been previously employed as an RCA.[44]

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[38] OPLR Rural Hospital Focus Group, July 2, 2025.
[39] OPLR Policy Scan
[40] OPLR Discussion with a member of the Kentucky Board of Respiratory Care, June 25, 2025.
[41] DOPL Complaint Data
[42] OPLR Interview Series
[43] OPLR Interview Series
[44] OPLR Discussion with a member of the Kentucky Board of Respiratory Care, June 25, 2025.

Finally, reinstating the RCA license would place only a modest burden on DOPL. Due to the limited timeframe in which someone can hold the license, the number of RCA licenses is expected to be low, and DOPL will not have to process renewals. Additionally, the costs associated with processing applications can likely be covered by the fee charged to the applicant. There will be some initial costs from establishing the license within DOPL's system, but this is expected to be marginal. Additionally, there would be costs associated with any investigations, but given that there were no complaints when this license was first issued, it is not expected that investigation costs will be significant.[45]

Other Considerations

Along with the recommendation above, OPLR considered the following policies:

Joining the Respiratory Care Interstate Compact

OPLR is neutral on the question of whether Utah should join the respiratory care interstate compact (see Appendix 5.1 for an expanded discussion). In its current form, the compact would allow Utah to continue licensing RCPs with a CRT credential, which OPLR feels is important for sufficient access. Otherwise, benefits to licensees are marginal. To practice in a member state, an RCP would still have to show that they meet that state's specific licensing requirements.[46] This is not drastically different from how states often deal with licensees moving from other states in the absence of a compact. The benefit of the compact is potentially faster application processes, improved data sharing between member states, and streamlined continuing education completion.[47]

If the compact were to require Utah to only licence RRT-credentialed RCPs, OPLR would recommend against joining it.

Creating an Advanced Practice Respiratory Therapy License

OPLR encourages the legislature to monitor the development of the advanced practice respiratory therapy (APRT) license in the U.S. (see Appendix 5.2 for an expanded discussion). Creating a group of respiratory care practitioners who could diagnose, treat, and prescribe in relation to cardiopulmonary issues could help fill pulmonary physician shortages and would give RCPs an opportunity to progress in their field.

However, the concept of an APRT license is still new, and widespread adoption would require employer interest, the creation of education programs, and a certification examination. This is a

[45] One DOPL administrator estimated a one-time cost of $750 to add the license into DOPL's system and set up electronic applications and a $1,250 annual cost for investigations.
[46] Respiratory Care Interstate Compact Model Language
[47] Correspondence with a member of the National Center for Interstate Compacts, June 6, 2025.

promising development in the field of respiratory care, and Utah may find it beneficial to pursue it in the future when these developments have taken place.

Requiring Continuing Education or Maintenance of CRT or RRT Credential for Renewal

OPLR does not recommend changes to the renewal requirements for RCPs.

As mentioned above, RCPs in Utah are not required to complete continuing education (CE) or show proof that their NBRC credential (CRT or RRT) is up to date.[48] There is variation in renewal requirements for health care professions in the state. For example, registered nurses (RNs) are required to do CEs if they have worked less than a certain amount, advanced practice nurses (APRNs) are required to maintain certification with an approved body, and physician assistants (PAs) must *either* maintain certification or do CEs. It is unique for a profession not to be required to do either of these things.[49]

However, OPLR ultimately decided against a recommendation to require CE or proof of continued credentialing at renewal for several reasons. First, unlike APRNs and PAs, RCPs cannot work independently, and hospital employers appear to already require continued NBRC credentialing from their RCPs.[50] Second, because RCPs mostly work in care teams within hospitals, they are often surrounded by practitioners who can help ensure safe practice. Finally, as mentioned previously, the low rate of DOPL complaints against RCPs indicates a lack of competency issues.

Rule Review

In accordance with Utah Code 13-1b-203(5), OPLR conducted an in-depth review of DOPL's respiratory care practitioner rules, found in R156-57.

The rule review covered potential rule changes needed to:

1. address specific rules that may be either overly burdensome (e.g., for individuals seeking to practice a profession or given the potential risk to public safety from a profession, etc) or insufficient (e.g., to ensure safe practice);
2. address rules misaligned with statutory language;
3. clarify language and correct references to statute or other rules; or
4. support OPLR's recommendations.

[48] "NBRC Credential vs State License Renewal," NBRC: NBRC requires renewal every five years. To renew an NBRC credential, an individual must either participate in continuing education, retake and pass their highest-level credential, or take the examination for a credential they do not yet hold.
[49] See R156-31b-303 & R156-70a-303
[50] OPLR correspondence with RCP leader at University of Utah Hospital.

OPLR's review of R156-57 found:

1. one rule for DOPL to monitor for potential burdens to RCPs. This is outlined in Appendix 6.1;
2. no rules misaligned with statutory language;
3. no unclear rule language but three incorrect references to statute. These are outlined in Appendix 6.2; and
4. new rules will need to be written to support a respiratory care apprentice license.

Appendix

Contents

1. Context
    - 1.1 General Methodology
    - 1.2 DOPL Respiratory Care Practitioner Licensee Renewal Survey
    - 1.3 Respiratory Care Literature and Policy Scans
2. Background
    - 2.1 Distinction Between CRT and RRT Credential
3. Regulatory Model Assessment & Recommendation
    - 3.1 Menu of Regulatory Models and Factors Considered in Framework
    - 3.2 Model Assessment of Respiratory Care
4. Regulatory Model Adjustments & Recommendations
    - 4.1 Possible Adjustments
    - 4.2 DOPL Complaint Data
    - 4.3 DOPL Licensee Data
5. Other Considerations
    - 5.1 Respiratory Care Interstate Compact
    - 5.2 Advanced Practice Respiratory Therapy License
6. Rule Review
    - 6.1 Potential Rule Burdens
    - 6.2 Incorrect References
7. Stakeholder Engagement
    - 7.1 OPLR Interview & Focus Group Series
    - 7.2 Stakeholder Engagement Summary
8. References

1 Context

1.1 General Methodology

OPLR's methodology combines qualitative and quantitative methods with robust stakeholder engagement. Methods include:

- Analyzing data from workforce surveys administered by the Department of Professional Licensing (DOPL) as part of licensure renewal
- Conducting quantitative analysis of DOPL licensee and complaint data and publicly available data from other state and federal government entities (e.g., Utah Department of Workforce Services, Health Resources and Services Administration, National Practitioner Database)
- Reviewing academic literature and reports on a profession's practice, efficacy and safety
- Scanning education and credentialing requirements, programs and content
- Reviewing state occupational regulation policies across the U.S.
- Engaging with a wide range of stakeholders, including: Utah and other state governments and agencies, industry organizations, researchers, practitioners, and business owners and employers within a variety of settings (e.g., acute inpatient hospital, private outpatient, hospital system outpatient, home health, and skilled nursing facilities)

1.2 DOPL Respiratory Care Practitioner Licensee Renewal Survey

Survey overview

OPLR utilized a DOPL survey available to respiratory care practitioners during their 2024 renewal period for information on the workforce in Utah. This survey is administered by DOPL for use by the Health Workforce Information Center (HWIC) to inform legislators and the public about workforce trends and projections. For more information regarding the information collected, the survey instrument can be found **here**.

Survey Limitations

The survey was available to all respiratory care licensees during the license renewal process so results were not affected by sampling bias. However, the response rate was relatively low for this survey, around 28%. Results may be affected by non-response bias (e.g., if those who chose to respond to the survey shared characteristics not representative of the true population). Respondents were more likely to be living in Utah and further into their career than non-respondents.

Other possible limitations include measurement error (which occurs when questions do not accurately measure the variable interest due to errors in question design) and recall bias (where respondents misremember and inaccurately answer questions). For example, recall bias may

impact the estimates of hours worked per week or debt at graduation. All of these potential errors may cause some variability or systematic bias.

OPLR uses this data to provide background understanding of a profession, outline patterns, and identify general trends rather than to provide exact estimates. Therefore, the limitations articulated above should not unduly impact OPLR's findings or recommendations.

1.3 Respiratory Care Literature and Policy Scans

To better understand the academic literature and regulatory environment for respiratory therapy, OPLR utilized the deep research function of Gemini (Google's AI tool). conducted a review of state occupational regulation in the U.S. OPLR analysts validated all results produced by Gemini. The validated sources were then used to evaluate research findings, map the national policy landscape, find patterns in regulation, make cross-state comparisons, and discover outliers. OPLR also used the data to help inform recommendations.

OPLR used the following basic prompt to search for literature:

"Are there any research papers that indicate that a respiratory therapist with only the CRT credential is less safe than a respiratory therapist with an RRT credential?"

OPLR verified all academic sources by scanning each source before utilizing them.

OPLR also used the deep research function of Gemini to determine how jurisdictions differ on their approach to very specific policies. Specifically, OPLR used Gemini to determine how many states require the RRT credential rather than the CRT credential, how many continuing education hours states require, and how many states have a respiratory care apprentice license. The following is an example of a prompt used to research state requirements for respiratory care licensure:

"*Will you look at each state to see what their licensing requirements are for respiratory therapists. Please put the information in the following table: column A contains the name of the state, column B indicates whether the state requires CRT, RRT, or either credential for licensure, column C indicates the number of required continuing education hours, and column D lists the hyperlinks to the sources you used to get the data. Please only use state statute or rule."*

OPLR analysts then verified Gemini's results for every state.

This approach does contain limitations related to normal human error. It is possible that there is slight misreporting of some data due to limited accessible state information, errors in data entry, or mistakes made by Gemini that were not caught by OPLR's manual verification.

2 Background

2.1 Distinction Between CRT and RRT Credential

The National Board for Respiratory Care (NBRC) administers a test called the Therapist Multiple Choice Examination, which includes 160 multiple choice questions. NBRC has set a 'low cut score' and a 'high cut score'.[51] Candidates who score above the low cut but below the high cut receive a CRT credential. Candidates who score above the high cut receive a CRT credential and also qualify for a second exam, the Clinical Simulation Examination, which includes 22 questions. Individuals who pass this additional exam receive an RRT credential.[52]

3 Regulatory Model Assessment & Recommendation

3.1 Menu of Regulatory Models and Factors Considered in Framework

Please see **this working document**, OPLR's Occupational Regulation Framework, for a more detailed explanation of OPLR's approach to assessing occupational regulation and evaluating different regulatory models.[53]

3.2 Model Assessment of Respiratory Care

The following table summarizes OPLR's analysis of respiratory care according to factors that OPLR determined should influence the appropriate regulatory model for an occupation. Factors that OPLR considered as particularly determinative in its assessment of respiratory care are highlighted in bold.

Model Assessment of Respiratory Care Practitioners (RCPs)	
Harm Factors	
Mechanism of Harm	RCPs typically operate ventilators to help patients breathe. The improper use of these machines could have life-threatening consequences.
Severity, Permanence, and Likelihood of Harm	**RCPs require specialized training to operate ventilators. An untrained individual would likely cause harm that could be severe and permanent, including death.**
Consequence of Error	**88 out of 100***
Downstream Impact	If a patient with a cardiopulmonary disorder, such as asthma or COPD, is treated inadequately by an RCP, their situation could

[51] "Certified Respiratory Therapist (CRT)", NBRC
[52] "Registered Respiratory Therapist (RRT)," NBRC
[53] The document is also available on OPLR's website in the "About OPLR" section, accessible here: https://oplr.utah.gov/about-oplr/

	require intensive medical attention.
Consumer & Setting Factors	
Patient Vulnerability	**RCPs work with patients who are highly vulnerable and either cannot breathe on their own or struggle to breathe without intervention.**
Frequency of Physical Touch	RCPs physically touch patients on a semi-frequent basis during procedures such as intubation or CPR.
Frequency of Private Setting	RCPs are typically part of a care team in hospital settings. They have, at most, short durations of private contact with patients.
Information Asymmetry	RCPs perform complex care tasks, such as the management of ventilators. A typical patient would not have the knowledge to evaluate how skilfully an RCP operates a ventilator.
Related factors	
Independence	RCPs are not sole practitioners. Additionally, in their setting, they must work under “medical direction or supervision.”**
Patient Choice	**Patients are assigned and do not choose their RCP.**
Information Availability	Because patients do not choose their RCP, there is little information available to patients about the quality of the RCP caring for them.
Level of Oversight	*Employers:* RCPs typically work for sophisticated employers who monitor the quality of their practice. At the same time, RCPs typically work without direct supervision and can work in more independent settings such as home health. *Private Bodies:* RCPs do have a private certifying body, the National Board for Respiratory Care (NBRC). While the NBRC requires continuous certification, it does not monitor behavior or revoke certifications from RCPs who provide inadequate care.***

* O*Net Consequence of Error Ranking
** UCA 58-57-12; UCA 58-57-2(3)(a)
*** National Board for Respiratory Care

4 Regulatory Model Adjustments & Recommendations

4.1 Possible Adjustments

Please see **this working document**, OPLR's Occupational Regulation Framework, for a more detailed explanation of how OPLR approaches whether adjustments should be made within a recommended regulatory model.[54]

4.2 DOPL Complaint Data

The Division of Professional Licensing (DOPL) receives complaints from individuals, other state agencies, co-workers, professional associations, and licensing boards. DOPL is required to "investigate unlicensed practice in regulated professions, acts or practices inconsistent with recognized standards of conduct, allegations of gross negligence or incompetence, and patterns of gross negligence or incompetence".[55] Violations that meet the criteria for investigation are then prioritized and assigned to an investigator. DOPL may resolve investigations in a variety of ways, including: closing an investigation due to a lack of evidence; referring the case to another agency or to law enforcement if appropriate; carrying out informal or formal administrative sanctions or stipulated agreements; issuing a citation; or denying, suspending, or revoking an individual's license.

To analyze complaints sent to DOPL, OPLR used My License Office (MLO) to access closed complaints investigated by DOPL between 2017-2022 for all licenses/professions. This data contains information on the license name, the complaint type, and the disposition of the complaint, among many other data fields not relevant to OPLR's analysis. DOPL personnel helped code the complaint dispositions as either substantiated, unsubstantiated, or no jurisdiction. Substantiated complaints are those where a disposition includes some type of disciplinary action, whether formal or informal (e.g., letter of concern, verbal warning, surrender of license). Unsubstantiated complaints have dispositions without a disciplinary action (e.g., dismissed, lack of evidence, unfounded). 'No jurisdiction' complaints are complaints that may or may not have basis, but DOPL was not able to take action on the case.

OPLR filtered complaints to exclude any likely duplicates and then used substantiated complaints to calculate the number of complaints per license type or profession. OPLR estimated the complaint rate for each license type by dividing the number of substantiated complaints by the number of unique individuals who held that license type over the same period.

[54] The document is also available on OPLR's website in the "About OPLR" section, accessible here: https://oplr.utah.gov/about-oplr/

[55] DOPL, An Explanation of the Complaint Handling Process for the Division of Occupational and Professional Licensing

Complaint Case Notes Analysis

A more detailed analysis of historical case notes was conducted on a sample of six respiratory care practitioner complaints closed between 2017-2022. A rough stratified random sample was pulled from all complaints categorized as substantiated or no jurisdiction,[56] with stratification based on complaint type. OPLR included the one complaint categorized as 'client harm' and the one complaint categorized as 'scope violation' as well as random samples of 'substance abuse related issues' and 'unprofessional conduct' complaints. OPLR then analyzed these cases and pulled specific information, such as whether a patient was harmed from the incident and how long the individual had been licensed when it occurred.

Limitations

There are significant limitations to this analysis, and the information collected should not be interpreted as a precise estimate of harm caused by respiratory care practitioners. DOPL data likely underestimates true harm, as many instances of harm may be handled in other ways (e.g., directly by employers), reported to other entities, or may never be reported. Additionally, some unsubstantiated complaints may have resulted in harm but the necessary evidence was not produced.

There could also be latent factors correlated with both the likelihood of complaint and the profession, systematically biasing the comparisons across professions. This is especially true in healthcare, as certain professions, by their nature, include a greater potential for harm and may generate more complaints. For example, surgeons have a higher likelihood of causing severe harm to a patient than respiratory care practitioners because surgery is inherently far riskier, not because surgeons are "less safe" or less competent than respiratory care practitioners.

For these reasons, OPLR uses DOPL complaint data as directionally informative, but avoids direct comparisons across professions wherever possible. Fine comparisons across professions are unwarranted and unsupported by this data.

4.3 DOPL Licensee Data

OPLR used DOPL licensee data queried in January of 2025 to conduct analyses on the number of licensees per year, inflow and outflow of licensees, overlap of licenses, and time with license. The dataset included individuals first licensed after 1970 to those actively licensed as of January 2025. Each row in this dataset was a unique combination of individual and license type and contained information regarding when the license was issued, the status of the license, the date the status was last updated, and the sex and year of birth of the individual. OPLR estimated the number of licensees in each year by summing the number of unique individuals whose licenses

[56] No jurisdiction complaints were included in the case note analysis and not the complaint rate analysis because they are not complaints where DOPL took an action, but they may include legitimate client harm that DOPL had to refer to another agency. Although they can not be classified as "substantiated", OPLR felt these complaints could help contextualize client harm resulting from respiratory care practitioners.

were active during any point in each year. Additionally, OPLR excluded any individual with a null or incorrect value for their license issue date and license expiration date, as OPLR could not determine how long or for what years they were actively licensed. License counts may slightly underestimate the true number of licensees due to this, but the effect is fairly negligible given OPLR's use of the data to determine trends over time rather than estimate with precision for specific dates.

5 Other Considerations

5.1 Respiratory Care Interstate Compact

An interstate compact for respiratory care has been written, and the American Association for Respiratory Care is working towards its adoption.[57] The compact has been adopted by four states so far and will be activated if and when it is adopted by three more states.[58]

5.2 Advanced Practice Respiratory Therapy License

The idea of creating an advanced practice respiratory therapy (APRT) license in the field is relatively new but gaining traction. No states have adopted the license type, but there is currently a bill in the Ohio legislature that would create this license, giving practitioners the ability to diagnose, treat, and prescribe non-controlled substances within the context of respiratory care under the supervision of a physician.[59]

The Ohio bill would require advanced practice respiratory care practitioners to complete a master's or doctorate degree, pass an examination approved by the board, and hold certification from an approved body.[60] The Ohio State University currently runs the only clinical respiratory care master's program that would qualify someone for this advanced license, though universities in other states are looking to create programs.[61] There are currently no examinations or credentials available for APRT, though NBRC is in the process of creating an exam and associated credential.[62]

There are several benefits that proponents of the APRT license articulate. First, an advanced license gives respiratory care practitioners (RCPs) an opportunity to progress within their profession and may decrease the number of RCPs becoming nurse practitioners (NPs), physician assistants (PAs) or leaving the field entirely.[63] Second, APRTs could help fill gaps

[57] "Respiratory Care Interstate Compact," AARC
[58] National Center for Interstate Compacts; Respiratory Care Interstate Compact Model Language
[59] H.B. 253, Ohio Legislature, 136th General Assembly Regular Session
[60] H.B. 253, Ohio Legislature, 136th General Assembly Regular Session
[61] OPLR Discussion with the Ohio State University Master of Respiratory Therapy Program Director, June 3, 2025; OPLR Discussion with a member of the Kentucky Board of Respiratory Care, June 25, 2025.
[62] OPLR Discussion with the Ohio State University Master of Respiratory Therapy Program Director, June 3, 2025
[63] OPLR Discussion with the Ohio State University Master of Respiratory Therapy Program Director, June 3, 2025. Among all RCPs licensed in Utah up until January 2025, only 3% later became NPs or PAs, although a slightly higher percentage of survey respondents report working outside of their field than

created by a shortage of pulmonary physicians in the same way NPs and PAs help fill physician gaps in areas such as primary care.[64]

6 Rule Review

6.1 Potential Rule Burdens

DOPL currently only approves education programs accredited by CoARC and only accepts examinations administered by the National Board for Respiratory Care (NBRC).[65] This is likely appropriate, as OPLR is not aware of other bodies that offer accreditation or examinations for respiratory care. However, DOPL should loosely monitor whether other organizations emerge that fulfill similar roles as effectively.

6.2 Incorrect References

OPLR identified the following incorrect references in R156-57:

Rule	Incorrect Reference	Correct Reference
R156-57-102(2)	58-57-2(4)(e)	58-57-3(b)(v)
R156-57-302a	58-57-4(2)(f)	58-57-4(2)(e)
R156-57-302b	58-57-4(2)(e)	58-57-4(2)(d)

7 Stakeholder Engagement

7.1 OPLR Interview & Focus Group Series

OPLR relied heavily on stakeholder engagement and qualitative interview data, combined with OPLR's other analysis, to conduct this review and develop recommendations. OPLR engaged with employers, educators, industry associations, Utah legislators, and Utah and other state regulators. OPLR prioritized diversity of perspective and relevance to the industry in selecting stakeholders.

Interviews were conducted in person, over the phone, and via video conferencing using semi-structured interview methods; they were conducted one-on-one and with multiple members. Extensive notes were taken for all interviews.

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similar healthcare professionals; OPLR Analysis of DOPL Licensee Data; DOPL Respiratory Care Practitioner Licensee Renewal Survey, July 2024.

[64] "Advanced Practice Respiratory Therapist," AARC; OPLR Discussion with the Ohio State University Master of Respiratory Therapy Program Director, June 3, 2025

[65] R156-57-302b; R156-57-302a

OPLR conducted initial interviews to understand the respiratory care industry, determine the largest issues related to safety and access, and identify potential areas for change. OPLR engaged with stakeholders later in its review to test initial findings from analysis and preliminary recommendations. OPLR reflected on and synthesized feedback across multiple discussion sessions to develop clear and achievable evidence-based recommendations.

Additionally, OPLR conducted six focus groups with administrators from various health care facility types: rural hospitals (eight administrators), rural health clinics (five administrators), ambulatory surgical centers (two administrators), skilled nursing facilities (four administrators), assisted living facilities (eight administrators), and home health and hospice companies (five administrators). Industry association leaders helped OPLR identify participants for these focus groups. Before the sessions, OPLR sent participants a short survey to identify any hiring and safety concerns for the nursing and related professions covered in this year's review (including respiratory care practitioners). During the focus groups, OPLR asked participants follow-up questions on these topics. Insights from the focus groups were used to direct OPLR's analysis and help refine recommendations.

Limitations

This interview sample was not randomly selected and, therefore, is not completely representative. OPLR spoke to individuals most likely to represent the broad aims and concerns of their groups. Additionally, OPLR did not contact "consumers" of respiratory care practitioners (patients), so their perspectives were not incorporated into this review. Thus, the stakeholder engagement and findings from these interviews should not be understood to be fully representative of the views of all Utahns, respiratory care practitioners, employers or any other person, group, or population.

Note that stakeholders' views are not always reflected in OPLR's recommendations. OPLR is directed by Utah Code 13-1b-302 to apply specific review criteria. These can and do lead to recommendations that diverge from stakeholder preferences. A stakeholder's appearance here is not an endorsement of OPLR's recommendations as such.

7.2 Stakeholder Engagement Summary

The following is a comprehensive list of those OPLR engaged with throughout the review of nursing and related professions. Professional credentials (such as MD and DNP) were omitted for simplicity. Those who contributed to the respiratory care review specifically are highlighted in gray.

Stakeholder Engagement Summary	
Utah State Legislature	
Legislative Working Group	**Rep. Bridger Bolinder** **Rep. Steve Eliason** **Rep. Katy Hall** **Rep. Cory Maloy** **Rep. Logan Monson** **Rep. Angela Romero** **Rep. Douglas Welton** **Sen. Luz Escamilla** **Sen. Keith Grover** **Sen. Ann Millner** **Sen. Jen Plumb** **Sen. Evan Vickers** **Seth Anderson,** Policy Analyst, LRGC **Brian Bean,** Senior Policy Advisor, Utah Senate **Greg Gun,** Associate General Counsel, LRGC **Alan Houston,** Associate General Counsel, LRGC **Tyler Moore,** Associate General Counsel, LRGC **Rohnin Randles,** Policy Analyst, LRGC **Lisa Sorenson,** Policy Analyst, LRGC **Chris Williams,** Associate General Counsel, LRGC **Robert Wood,** Policy Analyst, LRGC
Other Government Stakeholders	
Utah Department of Commerce	**Margaret Busse,** Executive Director **Carolyn Dennis,** Deputy Director **Jacob Hart,** Deputy Director **Mark Steinagel,** Managing Director & Director, Division of Professional Licensing **Deborah Blackburn,** Assistant Division Director, Division of Professional Licensing **Jana Johansen,** Assistant Division Director, Division of Professional Licensing **Connie Kitchens,** Assistant Division Director, Division of Professional Licensing **Benjamin Baker,** Chief Investigator, Division of Professional Licensing **Jeff Busjahn,** Licensing Administrator, Division of Professional Licensing **Camille Farley,** Chief Investigator, Division of Professional Licensing **James Garfield,** Bureau Manager, Division of Professional Licensing **Dean Healey,** Investigator, Division of Professional Licensing **Larry Marx,** Licensing Administrator, Division of Professional Licensing **Sharilee Scheller,** Investigator, Division of Professional Licensing **Kirsten Shumway,** Legal Analyst, Division of Professional Licensing
Division of Professional	**David Escobar,** Board Member, Respiratory Care Board

Licensing (DOPL) Boards	**William Hamilton,** Board Member, Medical Board **Erica Nelson,** Board Member, Nursing Board **Curtis Nielsen,** Board Member, Nursing Board **Marie Pittman Cherrington,** Medical Board **Ralph Pittman,** Board Member, Nursing Board **Wendy Rusin,** Board Member, Nursing Board **Sheryl Steadman,** Board Chair, Nursing Board **Shane Yardley,** Board Member, Nursing Board
Utah Department of Health & Human Services	**Nate Checketts,** Deputy Director **Heather Borski,** Assistant Deputy Director **Laurie Baksh,** Director, Office of Maternal & Child Health **Stacey Bank,** Medical Director **Kendyl Brockman,** Health Workforce Policy Analyst, Office of Primary Care & Rural Health **Jared Brown,** Program Manager, Office of Licensing **Trent Brown,** Assistant Director, Office of Reimbursement Continued Care & Audit **Matt Cottrell,** Research Analyst, Health Workforce Information Center **Liz Craker,** Health Program Coordinator, Office of Primary Care & Rural Health **John Curless,** Director, Office of Reimbursement, Coordinated Care & Audit **Jessica Fiedel,** Licensing Manager, Office of Licensing **Heather Flint,** Licensing Manager, Office of Licensing **Michelle Geller,** Policy Specialist, Office of Primary Care & Rural Health **Eric Grant,** Director, Office of Financial Services **Shanna Jaggers,** Research Consultant, Office of Research & Evaluation **Rick Little,** Director, Office of Research & Evaluation **Kyle Lunt,** Director, Office of Data, Systems & Evaluation **Stephanie McVicars,** Director, Early Hearing Detection & Intervention (EHDI); Cytomegalovirus (CMV) Public Health Initiative; Children's Hearing Aid Program (CHAP) **Ashley Moretz,** Director, Health Access Division **Nune Phillips,** Senior Policy Advisor **Mary Rindler,** Manager, Newborn Screening Program **Heather Sarin,** Quality Improvement Director, Utah Women & Newborns Quality Collaborative **Florencia Schapira De Grout,** Director, Office of Licensing **Suzanne Smith,** Coordinator, Perinatal Mortality Review **Jim Stamos,** Director, Office of Healthcare Policy & Authorization **Jennifer Strohecker,** Director, Division of Integrated Healthcare & State Medicaid **Heidi Sylvester,** Outreach Coordinator, Perinatal Mortality Review **Greg Trollan,** Director, Office of Managed Healthcare **Marc Watterson,** Director, Office of Primary Care & Rural Health **Anna West,** Workforce Development Coordinator, Office of Primary Care & Rural Health
Utah Department of Workforce Services	**Ben Crabbe,** Chief Economist **Dave Forgerty,** Assistant Director, Division of Workforce Research & Analysis

	Chris Williams, Director, Division of Workforce Research & Analysis
Government Employees from Other States	**Amanda Boulay,** Assistant Executive Director, Maine Board of Nursing **Emma Cozart,** Data Consultant, Washington Board of Nursing **Angela Duvall,** Manager for Health Education & Training Unit, Missouri Department of Health & Senior Services **Mary Sue Gorski,** Director of Research in Advanced Practice, Washington Board of Nursing **Ruby Grantham,** Manger, Laboratory & In-Home Services Unit, Florida Agency for Health Care Administration **Andrea Hager,** Program Information Manager, Maryland Prescription Drug Monitoring Program **Roberta Hills,** Program Director, Colorado Board of Nursing **Tamara McDaniel,** Executive Director, Kentucky Board of Respiratory Care **Patricia McNamee,** Nursing Practice Coordinator, Massachusetts Board of Nursing **Carol Moreland,** Executive Administrator, Kansas Board of Nursing **Sarah Wickenhagen,** Policy Analyst, Oregon State Board of Nursing
Industry Stakeholders	
Industry Associations	**Bonnie Baker,** Director of Professional Accountability, Utah Midwives Organization **Amy Bard,** Associate Executive Director, Utah Academy of Physician Assistants **Fara Bitter,** Vice President, Utah Midwives Organization **Michelle Buck,** APRN Senior Policy Advisor, National Council of State Boards of Nursing **Brittany Carver,** Executive Director, Utah Assisted Living Association **Liz Close,** Executive Director, Utah Nurses Association **Ranae Cowley,** Partner, Foxley & Pignanelli **Jon Cox,** Principal, Utah Public Affairs **Julia Dieperink,** State Policy Analyst, American Association of Nurse Practitioners **Lynsey Drew,** President, Utah Academy of Family Physicians **Leslie Fabian,** President, Utah Academy of Physician Assistants **Joan Gallegos,** Co-Lead, Utah Action Coalition for Health **Teresa Garrett,** Co-Lead, Utah Action Coalition for Health **Cheryl Gerdy,** President, Utah Organization of Nurse Leaders **David Gessel,** Executive Vice President, Utah Hospital Association **Francis Gibson,** President/CEO, Utah Hospital Association **Matt Hansen,** Executive Director, Homecare & Hospice Association of Utah **Bryanna Hazelwood,** President, Utah Midwives Organization **Melissa Hinton,** Interim President & Legislative Committee Chair, Utah Nurse Practitioner Association **Colby Jacobsen,** President, Utah Association of Nurse Anesthetists

	Tay Kopanos, Vice President of State/National Government Affairs, American Association of Nurse Practitioners **Viet Le,** Past President, Utah Academy of Physician Assistants **Nicole Livanos,** Director of Government Affairs, National Council of State Boards of Nursing **Stan Lockhart,** Executive Director, Utah Academy of Physician Assistants **Daniel Logsdon,** Director, National Center for Interstate Compacts, The Council of State Governments **Maryann Martindale,** CEO, Utah Academy of Family Physicians **Matthew McCullough,** Rural Hospital Improvement Director, Utah Hospital Association **Michelle McOmber,** CEO, Utah Medical Association **Abigail Mortell,** Policy Analyst, National Center for Interstate Compacts, The Council of State Governments **Miriam O'Day,** Senior Vice President, American Association for Respiratory Care **Michell Oki,** President, Utah Society for Respiratory Care **Ron Pasewald,** Co-Chair, Respiratory Care Interstate Compact **Camille Plowman,** Executive Director, Utah Association of Nurse Anesthetists **Nancy Sechrest,** Owner, Sechrest Consulting **Carl Sims,** Deputy Director, National Center for Interstate Compacts, The Council of State Governments **Daniel Sowards,** VP of Operations, Utah Health Care Association **Allison Spangler,** President/CEO, Utah Health Care Association **Matt Holton,** Partner, Stokes Strategies **Sherri Vasas,** Vice President, Utah Society for Respiratory Care **Sarah Woolsey,** Medical Director, Association for Utah Community Health; Speaker of the House, Utah Medical Association
Employers	**Larson Alder,** Executive Director, Symbii Home Health and Hospice **Greg Atwood,** VP of Clinical Operations, Avista Senior Living **Clint Barney,** Executive Director, Western States Lodging **Grant Barraclough,** Chief Clinical Officer, Ashley Regional Medical Center **Brenda Bartholomew,** Chief Administrative Officer, Gunnison Valley Hospital **Randall Bennett,** VP of Human Resources, Uintah Basin Medical Center **Heather Brace,** Chief HR Officer, Intermountain Health **Colten Bracken,** Owner, Main Street Family Medicine **Danelle Brinkerhoff,** Uintah Basin Medical Center **DeAnn Brown,** President, Garfield Memorial Hospital and Nursing Home **Jesse Buntjer,** VP of Clinical Operations, Avista Senior Living **Angela Chavez,** Student Programs Director, Intermountain Health **Amy Christensen,** Chief Nursing Operator, Intermountain Health Canyons Region **Jeff Christensen,** Executive Director, Millard Care & Rehabilitation **Julie Christensen,** Office Manager, Beaver and Parowan Medical Clinics **Dave Clarke,** Owner/COO, Covington Senior Living **Esmeralda Comer,** Practice Manager, Coalville and Kamas Health

	Centers **Reuben Evans,** Chief Nursing Officer, Intermountain Health St. George Hospital **Kurt Forsyth,** President, Delta and Fillmore Community Hospitals **Frank Fox,** CEO, Surgery Center at South Ogden, SCA Health **Nicholas Fox,** San Juan Health **Kris Friedli,** Nurse Administrator, Sunshine Home Health **Greg Gardiner,** Chief Nursing Officer, Uintah Basin Medical Center **Amanda Hartman,** Administrator, Uintah Basin Rehab and Senior Villa **Clayton Holt,** CEO, San Juan Health **Joleen Huber,** Owner, Caregiver Support Network **Kelly Jensen,** Assistant Vice President of Respiratory-Sleep-EEG Services, Intermountain Health Canyons, Desert, Peaks Regions **Ryan Kidman,** Administrator, The Lodge at Riverton and Jordan River **Shawn Kinross,** Advanced Practice Director of Anesthesia, Intermountain Health **Sulane Knight,** Chief Nursing Officer, Blue Mountain Hospital **Tiffiny Lipscomb,** VP of Human Resources, Intermountain Health **Kurt Loveless,** CEO, Kane County Hospital **Heather Mills,** Pediatric Director, Harmony Home Health and Hospice **Scott Monson,** CEO/Partner, SAL Management Group **Miranda Moss,** Executive Director, Legacy Village of Sugar House **Tracey Nixon,** Chief Nursing Officer, University of Utah Health **Kim Nuttal,** HR Director, Kane County Hospital **Jerry Olson,** Administrator, Southern Utah Veterans Home **Brett Ottley,** Executive Director, Monument Health **Susan Robel,** President, Canyons Region, Intermountain Health **Angel Roundy,** Chief Nursing Officer, Sevier Valley Hospital, Intermountain Health **Brent Schmidt,** President, Sevier Valley Hospital, Intermountain Health **Rachel Seamons,** Partner, Wasatch Senior Living **Will Shakespeare,** Senior Medical Director for Anesthesia, Intermountain Health **Beau Sorenson,** COO, First Choice Home Health and Hospice **Bethany Taylor,** Avista Senior Living **Shaylynn Uresk,** Director of Payroll, Uintah Basin Medical Center **Carrie Winberg,** Operations Director for Respiratory Care Services, Intermountain Health **Lori Wright,** CEO, Family Healthcare
Higher Education	**Jonathan Baird,** Assistant Program Director for the Physician Assistant Program, Rocky Mountain University **Jodie Buttars,** LPN Program Coordinator, Davis Technical College **Cherie Crezee,** Director, Utah Nursing Assistant Registry at Davis Technical College **Marla De Jong,** Dean, University of Utah College of Nursing **Eric Haskell**, LPN Program Coordinator, Bridgerland Technical College **Vic Hockett,** Associate Commissioner, Talent Ready Utah **Michelle Hofmann,** Senior Associate Dean, Spencer Fox Eccles School of Medicine, University of Utah

	Jane Lassetter, Dean, Brigham Young University College of Nursing **Timothy McCreary,** Psychiatry Concentration Track Director of Doctor of Medical Science, Rocky Mountain University **Rita Osborne,** Executive Director, Utah Center for Rural Health, Southern Utah University **Lauren Stanford,** Director of Didactic Education for the Physician Assistant Program, Rocky Mountain University **Carrie Torgersen,** Assistant Director, Utah Center for Rural Health, Southern Utah University
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Academics, Practitioners & Researchers	**Melanie Beagley,** Senior Health Research Analyst, Kem C. Gardner Policy Institute **Trent Bell,** Uintah Basin Medical Center **Kevin Byrne,** Huntsman Mental Health Institute, University of Utah **Erin Clark,** Associate Professor of Obstetrics & Gynecology,, Spencer Fox Eccles School of Medicine, University of Utah **Kirsten Corazzini,** Dean, College of Health & Human Services, University of New Hampshire **Tim Dall,** Managing Director for Life Sciences Consulting, GlobalData **Jessica Davidson,** Assistant Professor in Neonatology, University of Utah and Co-Medical Director of NICU at Intermountain Medical Center **Trish Hentre-Barrus,** Riverwoods Behavioral Health **Lydia Howes,** Assistant Librarian, Eccles Health Sciences Library, University of Utah **Louise Kaplan,** University of Washington Center for Health Workforce Studies **Sri Koduri,** Consultant, SiriS Consulting **Benjamin Lewis,** Huntsman Mental Health Institute, University of Utah **Jonathan Martin,** Intermountain Health **Carissa Monroy,** Sacred Circle Healthcare **Christine Mueller,** University of Minnesota (retired) **Patricia Pittman,** Director, Fitzhugh Mullan Institute for Health Workforce Equity, George Washington University **Benjamin McMichael,** Associate Professor, The University of Alabama School of Law **John Poelman,** Director of Innovation, One Utah Health Collaborative **Amber Rasmussen,** Sacred Circle Healthcare **Brett Richards,** Orthopedic Surgeon, Ogden Clinic **Lee Rogers,** Community Health Centers **Clark Ruttinger,** Health Workforce Consulting Director, GlobalData **Nena Schvaneveldt,** Associate Librarian, Eccles Health Sciences Library, University of Utah **Georgianna Sergakis,** Director, Master of Respiratory Therapy Program, The Ohio State University **Laura Summers,** Director of Industry Research, Kem C. Gardner Policy Institute **Matthew Wells,** South Market OBGYN Medical Director, Canyon Region, Intermountain Health

	Jamie Wissler, Executive Director, One Utah Health Collaborative

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